



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Mental State Deterioration and Escalation Care Pathway (Adult)

Reference #: FSFH-HW-POL-0081

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Hospital Wide	Medical, Nursing
Exclusions: • Patients admitted with a new acquired brain injury requiring impaired cognition observations or requiring Post Traumatic Amnesia (PTA) testing. • Patients admitted in Mental Health areas. • Patients on an Eating Disorder Patient Care Plan. • Patients admitted under Maternity Services.		

1. Introduction

The purpose of this document is to:

- provide support to clinical staff in using the Mental State Deterioration and Escalation Care Pathway.
- assist staff to identify, respond to, and escalate acute changes in a patient's mental state using the Mental State Deterioration and Escalation Care Pathway.
- enable staff to anticipate factors that may contribute to deterioration in a person's mental state and where possible, modify these factors to prevent deterioration.
- provide baseline information to assist staff in recognising deviations from individual patient behaviours noting that what is considered typical behaviour for one person may be highly unusual for another person.

Further information relating to assessment and management of deterioration of a person's mental state can be found in the "Prevention and Management of Unpredictable Behaviours"

2. Terminology

WOKE	An assessment tool formulated to assist staff to identify, evaluate and escalate changes in a patient's mental state. The WOKE acronym represents the assessment system and comprises of: • What is the baseline mental state; assess and enquire • Observe and Engage • Know what to do; know who to refer to • Evaluate
Mental state	A person's intellectual capacity, emotional state, and general mental health based on clinical observations and interviewing. Mental state comprises of mood, behaviour, orientation, judgment, memory, problem-solving ability, and contact with reality.
Mental State Deterioration	Changes in a person's mental state that indicate the need for closer observation, clinical review and / or more frequent review, and for the introduction, change or 'up-scaling' of therapeutic interventions.
Reported change	When a person, or someone who knows the person well, reports that their mental state has changed.
Loss of function	A reduction in a person's ability to think clearly, communicate, or engage in regular activities.
Safety Plan	A structured plan developed collaboratively between Clinicians, the consumer and their family, and their carers that can be worked through when the consumer is experiencing thoughts or feelings about violence or aggression, distress or crisis or following a violent or aggressive incident.

3. Policy

Members of the nursing and medical workforce must:

- Recognise deterioration in a person's mental state
 - Use the Mental State Deterioration and Escalation of Care Pathway when indicated on presentation, during clinical examination and throughout history taking or if indicated during the period of admission.
 - Escalate care in steps consistent with the acuity of deterioration in a person's mental state using the Mental State Deterioration and Escalation Care Pathway as a guide. The pathway defines timeframes for escalation, depending on the level of risk assessed.

- Respond to deterioration in a person's mental state
 - When early warning signs of deterioration in a person's mental state are observed or reported, members of the healthcare team will determine the immediate actions to prevent further deterioration.
 - Be aware of and use the Mental State Deterioration and Escalation Care Pathway.

4. Procedure

4.1. Nursing Admission and Assessment

4.1.1. Initial Screening Questions

- During every nursing admission, screening questions are completed on the Nursing Admission and Assessment e-Form including:
 - Gather baseline information on admission and establish a person's current mental state.
 - If the assessment determines a "yes" to any of the initial screening questions, the mental state deterioration and escalation of care pathway is to be employed.
 - If the assessment determines "no" to all initial screening questions the mental state deterioration and escalation of care pathway is not indicated.
- Refer to Figure 1 for screening questions, located in the Nursing Admission and Assessment E-Form – Mental Wellbeing section.

Figure 1: Nursing Admission screening questions

Admitted to a Mental Health setting? Yes / No

Mental State features exhibited.

- ☐ Distress
- ☐ Loss of touch with reality or consequences
- ☐ Loss of mental function
- ☐ Elevated risk to self or others
- ☐ NO mental state features exhibited.

[If any (except for NO mental state features) selected] *Action: Refer to Mental State Deterioration and Escalation of Care Pathway*

1. **Do you have a pre-existing mental health diagnosis?** Yes / No / Unable to assess.
If Yes or Unable to assess: Specify.
2. **Are you receiving treatment for your mental health?** Yes / No / Unable to assess.
If Yes: Specify (e.g., medications or other therapies)
Action: Refer to Mental State Deterioration and Escalation of Care Pathway
3. **Are you, your carer or next of kin concerned that your mental state has deteriorated in the last 7 days?** Yes / No / Unable to assess.
Action: Refer to Mental State Deterioration and Escalation of Care Pathway

4.2. Deterioration and Escalation of Care Pathway

4.2.1. Assess and Enquire

- In the initial screening, identify the need to perform the mental state deterioration and escalation of care pathway.
- Baseline information is an essential component to determining whether deterioration has occurred – this must be established and documented in the DMR. To establish the patient's baseline mental state, assess and enquire about the following:
- Assess for:
 - Reported change in behaviour
 - Distress
 - Loss of touch with reality or consequence of behaviours
 - Loss of function, and
 - Elevated risk to self, others or property.
 - Enquire with the patient, as well as their family, carer and/or Next of Kin (NOK) to determine whether they have observed changes in behaviour consistent with deterioration in the patient's mental state. Families and close friends have a central role in identifying early stages of deterioration in a person's mental state.
- If the patient does not exhibit any of the features described above, document their baseline mental state in the DMR and discontinue the pathway.
- Reassess the patient using the pathway (refer to [Figure 2](#)) next shift.

If the patient displays any of the indicators listed above, or the patient, family, carer or NOK report the potential for deterioration, commence safety planning and continue through the Mental State Deterioration and Escalation Care Pathway; observe and engage.

4.2.2. Observe and Engage

- Observe the patient's behaviour for:
 - threats to others
 - physical or verbal aggression
 - expression of self-harm or suicidal behaviour
 - agitation or restlessness
 - bizarre behaviour
 - withdrawn behaviour

- pessimistic expression or
 - ambivalence regarding treatment.
- Utilising the Mental State Deterioration and Escalation Care Pathway perform the appropriate escalation action:
 - If the patient has a known Early Warning Score/Adult Deterioration Detection Score for their physiological observations, escalate care as guided to do so in the FSFHG Observation and Response Chart policy and procedure.
 - Nursing staff are required to refer to FSFHG Escalation process for nursing staff
 - Address potential underlying physiological cause, where possible.
 - Engage with the:
 - Patient
 - Family/Carer/NOK
 - Treating team
 - Self
 - Have you/they observed any of these behaviours?
 - Are you/they concerned?

4.2.3. Know what to do and know who to refer to

Know what to do:

- Follow the Mental State Deterioration and Escalation Care Pathway.
- Potential interventions include:
 - Consideration of 1:1 care (refer to FSFHG Specialising and observations: Provision of a special for patients in non-psychiatric clinical areas)
 - Assess for delirium (perform 4AT)
 - Assess for drug or alcohol withdrawal (refer to FSFHG Alcohol and other drugs management of addiction, toxicity and withdrawal)
 - Address cultural needs, incorporate translation services if applicable
 - Relocation to a calm or low stimulus environment
 - Place patient in visible location
 - Perform environmental checks
 - Remove any potential harm or weapons.
 - Utilise the FSFHG Management of Agitation in Older Adults with Dementia or Delirium guideline.

- Document all actions:
 - In the Patient Care Plan
 - within the integrated progress notes in the DMR
- Include this information in handover and the Deterioration Intervention Concern Exposure (DICE) huddle.

Know who to refer to:

- Ward NUM or HOOT CNM after hours
 - MH HOOT CNM
- Mental Health Liaison service
 - Adults + Adolescents
 - Older adults + SRS patients
 - Perinatal
 - Drug and alcohol services
- Aboriginal Health Liaison Officer.

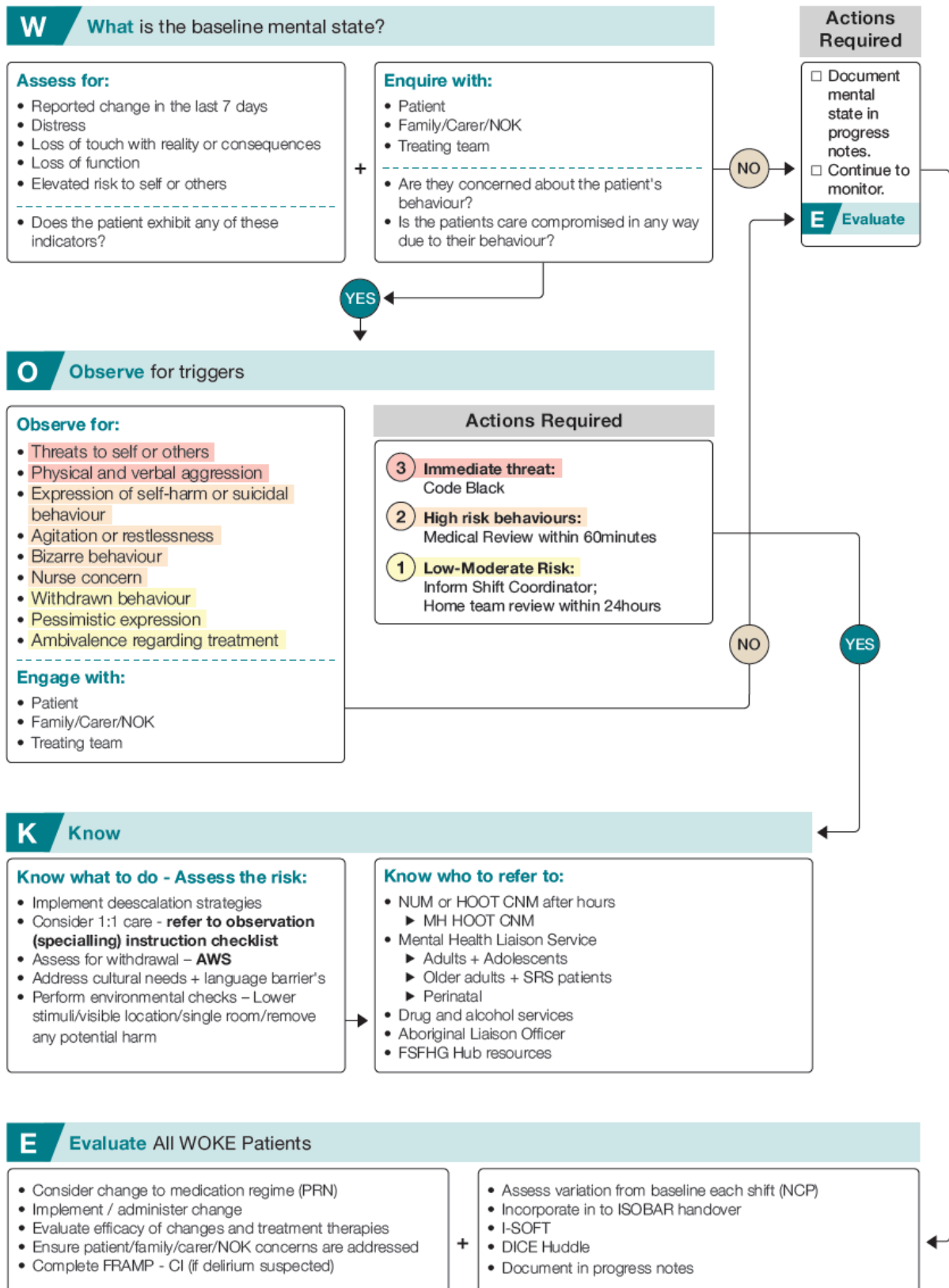
Utilise the tool within Figure 2, to guide the time frame of your escalation and facilitate timely intervention. The prevention of escalating behaviour is the best outcome for both patient and staff.

4.2.4. Evaluate

- Document Safety Plan for the patient including:
 - Assess and consider changes to the patient's medication regime
 - Use additional medication to manage behaviour if required
 - Implement and administer appropriate changes
 - Re-evaluate efficacy of interventions by performing WOKE each shift
 - Ensure that the patient, family, NOK, and/or carer concerns are felt addressed
- Assess the patient's mental state variation from baseline each shift
 - Incorporate in to Identify, Situation, Observation, Background, Action and Response (ISOBAR) handover
 - Document in i.Clinical Manager
 - Include in DICE Huddle
 - HOOT to flag as Patient of Concern on SHIMS.

Figure 2: The Mental State Deteriorating and Escalation of Care Pathway

Mental State Deterioration & Escalation of Care Pathway



Patients identified as requiring the Mental State Deterioration and Escalation Care Pathway are to be re-assessed each shift. Update the Patient Care Plan (refer to Figure 3) to reflect this.

Figure 3: Documentation within the Patient Care Plan

SHIFT BY SHIFT CHECK																								
Instructions: Please date and initial below to confirm which interventions are implemented each shift.																								
Document Baseline Mental State:																								
Date																								
Shift	A	P	N	A	P	N	A	P	N	A	P	N	A	P	N	A	P	N	A	P	N			
W																								
O																								
K																								
Escalation Action																								
E																								

- Document the level of assessment put into place each shift, by dating and initialling the relevant letter on the shift by shift check. Only initial against the level of intervention.
 - E.g. if the patient only requires 'W' and 'E' but does not progress to 'O' and 'K', initial against 'W' and 'E' only.
 - If an escalation occurs, e.g. an MO is contacted to review within 60minutes, or a CODE BLACK initiated, document the corresponding 'Action' number in the 'Escalation Action' box.

5. Compliance/Performance Monitoring

The FSFHG Recognising and Responding to Acute Deterioration Committee is responsible for reporting and monitoring compliance with this guideline.

6. Related Policy Documents

FSFHG:

- Prevention and Management of Unpredictable Behaviours (FSFH-MENH-POL-0026)
- Escalation process for nursing and Midwifery staff (FSFH-HW-POL-0014)
- Specialising and observations: Provision of a special for patients in non-Psychiatric clinical areas (FSFH-HW-GUI-0045)

- Observation and Response Chart (FSFH-HW-POL-0032)
- Alcohol and Other Drugs management of addiction, toxicity and withdrawal (FSFH-HW-POL-0063)
- Management of Agitation in Older Adults with Dementia or Delirium (FSFH-HW-GUI-0006)
- Cognitive screening of the older person (FSFH-HW-POL-0028).

7. Related Standards

- NSQHS Standards:
 - Clinical Governance
 - Partnering with Consumers
 - Comprehensive Care
 - Communicating for Safety
 - Recognising and Responding to Acute Deterioration.

8. Bibliography

1. Craze L, McGeorge P, Holmes D, Bernardi S, Taylor P, Morris-Yates A, et al. Recognising and responding to deterioration in mental state: A scoping review. Sydney: ACSQHC, 2014.
2. Trzepacz PT, Baker RW. The psychiatric mental status examination. New York: Oxford University Press; 1993.
3. Gaskin C, Dagley, G. Recognising Signs of Deterioration in a Person's Mental State. Sydney: ACSQHC, 2018.
4. Craze L, McGeorge P, Holmes D, Bernadi S, Taylor P, Morris-Yates A, et al. Recognising and responding to deterioration (2nd ed.). Sydney: ACSQHC, 2017.

9. Authorisation

EXECUTIVE SPONSOR: Medical Director, Service 4					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	09/2021	MET Coordinator	RRAD, NMPGC	FSFHG Policy Committee	09/2024
1.1	10/2021	MET Coordinator	RRAD	RRAD	09/2024
1.2	11/2021	MET Coordinator		RRAD	09/2024
4	08/2024	MET CNS	RRAD	FSFHG Policy Committee	10/2028